NCE EXAM PREP

Practice Exam 26

Domain 2

Intake & Assessment

30 Practice Questions | 40 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 05. Use the suggested 40-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

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PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> At intake, a counselor knows the counselor will leave the agency in five weeks. The client is seeking long-term counseling, and the agency can offer either short-term work with this counselor followed by transfer or immediate assignment to another clinician.

What should the counselor do?

A) Disclose the anticipated departure and explain the available continuity options before the client chooses how to begin

B) Conceal the departure until the final session so the client does not decline services

C) Promise long-term availability even though the counselor knows that promise cannot be kept

Question 2

> A prospective client requests treatment for a complex eating disorder and assumes the assigned counselor is an eating-disorder specialist. The counselor has generalist training but lacks the competence and supervision needed for this presentation, while qualified specialty providers are available locally.

What is the best intake action?

A) Explain the counselor's scope and facilitate referral to a qualified specialist rather than implying expertise the counselor does not have

B) Accept the case and learn the specialty through trial and error during sessions

C) Avoid discussing qualifications because clients should not influence provider selection

Question 3

> For three months, a client has experienced recurrent unexpected surges of intense fear with palpitations, sweating, trembling, shortness of breath, and fear of dying. After the first episode, the client spent more than a month worrying about another attack and stopped exercising to avoid triggering sensations. Medical and substance causes have been ruled out.

Which diagnosis is best supported?

A) Specific phobia

B) Panic disorder

C) Generalized anxiety disorder

Question 4

> A client has slept about two hours total in six days, speaks continuously, expresses grandiose beliefs, has spent the family's savings, and has driven at extreme speeds. The client rejects outpatient help and says ordinary safety limits do not apply.

Which disposition is most appropriate?

A) Schedule routine weekly counseling because high energy alone does not affect level of care

B) Arrange urgent psychiatric evaluation at a level of care able to address severe impairment and immediate safety

C) Wait for a self-report personality inventory before determining whether intervention is needed

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Question 5

> A client spends several hours daily examining a slight facial asymmetry that others barely notice, repeatedly seeks reassurance, and avoids work meetings because of shame. The concern is not focused on body weight or fat.

Which diagnosis is most consistent with the presentation?

A) Illness anxiety disorder

B) Anorexia nervosa

C) Body dysmorphic disorder

Question 6

What is a ceiling effect in an assessment?

A) Scores cluster near the bottom because items are too difficult for the group

B) Scores become more reliable as the number of items decreases

C) Scores cluster near the top, limiting the measure's ability to distinguish high levels of the attribute

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Question 7

> A client begins intake by saying, “Just tell me what decision to make about ending my relationship. I expect the counselor to choose for me.” The client understands that counseling is voluntary but misunderstands the counselor's role.

What is the best response?

A) Make the decision immediately so the client knows what counseling can accomplish

B) Refuse services because wanting advice makes the client unsuitable for counseling

C) Clarify the counselor's collaborative role and explore the client's values, goals, and expectations before proceeding

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Question 8

> An older adult says a live-in caregiver pressures the client to sign checks, withholds transportation when the client refuses, and has recently taken control of the client's bank card. The client appears frightened but can describe the events clearly and asks what help is available.

What is the counselor's best assessment response?

A) Assume incapacity because the client is older and ask the caregiver to explain the finances

B) Assess immediate safety, coercion, access to basic needs, decision-making ability, and the client's wishes, then follow applicable protective procedures

C) Treat the issue solely as a family disagreement unless physical injury is visible

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Question 9

> A counselor is selecting a symptom measure for a client with severe visual impairment. A validated audio-administered version is available, while the client's relative offers to read and rephrase the printed items.

Which approach is most appropriate?

A) Ask the relative to simplify any item that seems confusing

B) Omit formal assessment because visual impairment makes standardized measurement invalid

C) Use the validated accessible version according to its administration instructions and integrate the results with other assessment data

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Question 10

What does a correlation of −0.70 between two assessment variables indicate?

A) The variables have no relationship

B) Higher values on one variable tend to be associated with lower values on the other

C) One variable causes the other to decrease by 70%

Question 11

> A client reports intense inner restlessness and an inability to sit still that began three days after an antipsychotic dose was increased. The client paces throughout the interview and says the sensation feels different from prior anxiety.

What is the best assessment action?

A) Diagnose panic disorder because motor restlessness is a panic symptom

B) Interpret the pacing as intentional resistance to the interview

C) Assess a possible medication-related movement effect and coordinate prompt review with the prescribing clinician

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Question 12

A client says a sibling could provide useful history but does not want the counselor to contact the sibling yet. The counselor believes collateral information might eventually help but can begin the intake using the client's report and other available information.

What should the counselor do?

A) Contact the sibling privately because clinical usefulness overrides the client's preference

B) Respect the client's current decision, explain the possible value and limits of collateral information, and revisit permission if appropriate

C) Refuse to continue the intake until the client authorizes the sibling interview

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Question 13

An online intake form automatically displays preselected consent boxes for email reminders, information sharing with a family contact, and participation in optional workshops. The client has not reviewed the selections.

What should the counselor do?

A) Review each choice with the client and obtain affirmative, informed decisions rather than relying on default selections

B) Treat every preselected box as valid because the client opened the form

C) Leave the boxes unchanged and explain them only if the client later complains

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Question 14

Which statistic describes the difference between the highest and lowest scores in a distribution?

A) Variance

B) Standard deviation

C) Range

Question 15

At a couple's intake, one partner asks whether the counselor will become that partner's individual counselor if the relationship ends. The practice policy states that the couple is the client and that either partner seeking later individual work will be referred to another clinician.

What should the counselor do?

A) Promise to choose whichever partner requests individual counseling first

B) Delay discussion of future role changes until the couple separates

C) Explain the defined client, the policy on later individual services, and the available referral process before obtaining consent

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Question 16

> A 10-year-old has severe temper outbursts at least three times weekly and remains persistently irritable between outbursts. The pattern has lasted 14 months, occurs at home and school, began before age 10, and has never included a distinct manic or hypomanic episode.

Which diagnosis is most consistent with these facts?

A) Bipolar I disorder

B) Disruptive mood dysregulation disorder

C) Intermittent explosive disorder

Question 17

> A client has recurrent major depressive episodes. During one episode, delusions and hallucinations were present, and psychotic symptoms also persisted for three weeks when no major mood episode was present. Mood episodes have occupied most of the total illness duration, and substances and medical causes are excluded.

Which diagnosis is most consistent with this longitudinal pattern?

A) Major depressive disorder with psychotic features

B) Schizoaffective disorder, depressive type

C) Brief psychotic disorder

Question 18

> A 16-year-old has been distractible and impulsive since early childhood across home and school. Twice in the past year, the adolescent also had distinct five-day periods of decreased need for sleep, expansive mood, pressured speech, and markedly increased goal-directed activity.

What is the best assessment interpretation?

A) The episodic mood changes prove that the longstanding attention symptoms were never clinically relevant

B) Assess a possible attention disorder and a co-occurring bipolar-spectrum condition because chronic and episodic symptoms may represent different processes

C) Diagnose only an attention disorder because bipolar symptoms cannot occur in adolescence

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Question 19

A prospective client sends the counselor a friend request on a personal social media account and assumes online contact will be part of counseling. The practice has a written policy against personal social media connections with clients and uses a separate professional page for general announcements.

What should the counselor do during intake?

A) Accept the request temporarily so the client does not feel rejected

B) Ignore the issue because social media expectations are unrelated to counseling consent

C) Explain the social media policy, its privacy and boundary rationale, and the approved ways to communicate with the practice

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Question 20

A cognitive assessment yields average verbal reasoning, very low processing speed, and high-average visual-spatial scores. The client asks whether the overall composite means every area of thinking is average.

How should the counselor explain the findings?

A) Describe the meaningful variation among domains, the limits of the composite, and how results fit with history and functioning

B) Report only the composite because differences among domain scores are clinically irrelevant

C) State that the lowest domain alone represents the client's overall cognitive ability

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Question 21

Evidence that a new anxiety measure correlates strongly with established anxiety measures but weakly with unrelated physical-strength measures most directly supports which property?

A) Test-retest reliability

B) Construct validity

C) Interrater reliability

Question 22

During a mental status examination, the counselor asks a client to recite the months of the year backward. The client understands the task but repeatedly loses place and cannot complete it.

Which mental status function is this task primarily sampling?

A) Attention and concentration

B) Long-term autobiographical memory

C) Insight into illness

Question 23

> A rural client has unstable broadband and cannot reliably use the clinic's secure video platform. Clinic policy permits telephone counseling when the counselor determines it is clinically suitable, and an in-person site is available twice each month.

What is the best intake response?

A) Close the referral because unreliable broadband proves that remote counseling cannot work

B) Promise telephone counseling without assessing privacy, clinical needs, or emergency access

C) Review the client's technology, privacy, clinical needs, and available formats, then determine whether telephone, periodic in-person, or another referral is suitable

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Question 24

Which procedure provides an estimate of split-half reliability?

A) Comparing test scores with a later external outcome

B) Having two clinicians independently rate one interview

C) Correlating scores from two comparable halves of the same instrument

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Question 25

> A counseling practice offers optional animal-assisted sessions as well as sessions without an animal. A new client has a severe fear of dogs and believes declining animal involvement will make the client ineligible for care.

What should the counselor clarify?

A) Animal-assisted work is optional, its relevant benefits and risks, and the available counseling alternative without an animal

B) Exposure to the dog is required before the client's fear can be assessed

C) The client must accept every modality offered by the practice to receive counseling

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Question 26

> A new client asks the counselor to delete every note immediately after each session. Clinic policy requires records to be maintained for a stated retention period and permits clients to request access or correction through specified procedures.

What is the most appropriate intake response?

A) Explain the record-retention policy, what is documented, applicable access and correction procedures, and the limits of the client's deletion request

B) Promise immediate deletion and disregard the clinic policy once counseling begins

C) Refuse to discuss records because documentation practices are not part of informed consent

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Question 27

> A physician referred a client for stress management, but the client says the main concern is grief and is unsure what the physician expects the counselor to report. No release of information has been signed.

What should the counselor do during intake?

A) Explore the client's own concerns and goals, clarify the referral and possible communication, and obtain specific permission before contacting the physician

B) Adopt stress management as the treatment goal because the physician initiated the referral

C) Send a complete intake summary to the physician before asking the client about disclosure preferences

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Question 28

> An 8-year-old speaks freely with family at home but has not spoken at school for four months, despite understanding the language used there. The child communicates nonverbally with classmates, appears anxious when expected to speak, and has no speech disorder or unfamiliarity with the school language.

Which diagnosis is most consistent with the presentation?

A) Language disorder

B) Selective mutism

C) Autism spectrum disorder

Question 29

In an approximately normal distribution, about what proportion of scores falls within one standard deviation above or below the mean?

A) 68%

B) 50%

C) 95%

Question 30

> A client repeatedly picks at minor skin irregularities, causing lesions and scarring. The client has made several unsuccessful attempts to stop, spends substantial time concealing the damage, and experiences significant distress. The behavior is not due to a substance, medical condition, or another mental disorder.

Which diagnosis is best supported?

A) Excoriation disorder

B) Body dysmorphic disorder

C) Obsessive-compulsive personality disorder

PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: A

Correct Answer: Disclose the anticipated departure and explain the available continuity options before the client chooses how to begin

Explanation: The known five-week limit is material to a client seeking long-term care. Early disclosure permits an informed choice between short-term work with transfer and immediate assignment to a clinician who can provide continuity.

Question 2 — Correct: A

Correct Answer: Explain the counselor's scope and facilitate referral to a qualified specialist rather than implying expertise the counselor does not have

Explanation: The requested presentation exceeds the counselor's stated competence, and qualified specialty care is available. Accurate role disclosure and referral protect service quality without misleading the client about expertise.

Question 3 — Correct: B

Correct Answer: Panic disorder

Explanation: The client has recurrent unexpected panic attacks followed by more than one month of persistent concern and maladaptive behavioral change. The pattern is not limited to one feared object and is not the broad chronic worry of generalized anxiety disorder.

Question 4 — Correct: B

Correct Answer: Arrange urgent psychiatric evaluation at a level of care able to address severe impairment and immediate safety

Explanation: Profound sleep loss, grandiosity, financial devastation, dangerous driving, and refusal of limits indicate a severe manic presentation with acute safety concerns. Routine counseling or delayed testing would not provide an adequate level of care.

Question 5 — Correct: C

Correct Answer: Body dysmorphic disorder

Explanation: The client is preoccupied with a slight or unobservable appearance flaw, performs repetitive checking and reassurance seeking, and has marked impairment. The concern is not about illness or body fat and therefore does not fit the distractor diagnoses.

Question 6 — Correct: C

Correct Answer: Scores cluster near the top, limiting the measure's ability to distinguish high levels of the attribute

Explanation: A ceiling effect occurs when a measure is too easy or has an upper limit that compresses high scores. This reduces discrimination among people at the upper end of the construct.

Question 7 — Correct: C

Correct Answer: Clarify the counselor's collaborative role and explore the client's values, goals, and expectations before proceeding

Explanation: The client's request reflects a correctable misunderstanding about the counseling role rather than automatic unsuitability. Clarification allows intake to proceed without the counselor taking over a personal decision.

Question 8 — Correct: B

Correct Answer: Assess immediate safety, coercion, access to basic needs, decision-making ability, and the client's wishes, then follow applicable protective procedures

Explanation: The report suggests possible financial exploitation and coercive control that may affect safety and basic access. Age alone does not establish incapacity, and visible physical injury is not required before assessing and responding to the concern.

Question 9 — Correct: C

Correct Answer: Use the validated accessible version according to its administration instructions and integrate the results with other assessment data

Explanation: An established accessible version preserves standardization better than ad hoc rephrasing by a relative. Visual impairment does not make all formal assessment invalid when an appropriate administration method exists.

Question 10 — Correct: B

Correct Answer: Higher values on one variable tend to be associated with lower values on the other

Explanation: A negative correlation indicates an inverse association, and a magnitude of 0.70 suggests a relatively strong relationship. Correlation does not establish that one variable causes a specified percentage change in the other.

Question 11 — Correct: C

Correct Answer: Assess a possible medication-related movement effect and coordinate prompt review with the prescribing clinician

Explanation: The close onset after a dose increase, subjective inner restlessness, and visible pacing suggest possible akathisia. Treating the behavior as panic or resistance would miss a potentially significant medication effect.

Question 12 — Correct: B

Correct Answer: Respect the client's current decision, explain the possible value and limits of collateral information, and revisit permission if appropriate

Explanation: Potential clinical usefulness does not override the adult client's current choice about routine collateral contact. Intake can proceed using available information while the counselor explains why the subject might be revisited.

Question 13 — Correct: A

Correct Answer: Review each choice with the client and obtain affirmative, informed decisions rather than relying on default selections

Explanation: Prechecked boxes do not demonstrate that the client understood or actively selected each optional disclosure or service. The counselor should separate the choices and obtain meaningful consent for each.

Question 14 — Correct: C

Correct Answer: Range

Explanation: The range is calculated from the highest and lowest scores. Variance and standard deviation instead summarize dispersion using all scores in the distribution.

Question 15 — Correct: C

Correct Answer: Explain the defined client, the policy on later individual services, and the available referral process before obtaining consent

Explanation: The practice's handling of role transitions is material to informed consent in couple work. Promising future individual treatment to one partner would conflict with the stated policy, while delaying disclosure prevents informed agreement.

Question 16 — Correct: B

Correct Answer: Disruptive mood dysregulation disorder

Explanation: Severe recurrent outbursts plus persistent between-outburst irritability across settings for more than a year fit disruptive mood dysregulation disorder in the stated age range. The absence of distinct manic or hypomanic episodes argues against bipolar I disorder.

Question 17 — Correct: B

Correct Answer: Schizoaffective disorder, depressive type

Explanation: The history includes psychosis for at least two weeks outside a major mood episode, while depressive episodes occupy most of the illness duration. Psychosis limited to depression would instead support major depressive disorder with psychotic features.

Question 18 — Correct: B

Correct Answer: Assess a possible attention disorder and a co-occurring bipolar-spectrum condition because chronic and episodic symptoms may represent different processes

Explanation: Longstanding cross-setting attention symptoms and distinct episodic mood changes have different time courses and may coexist. Neither history should automatically erase the other, and adolescence does not preclude bipolar-spectrum assessment.

Question 19 — Correct: C

Correct Answer: Explain the social media policy, its privacy and boundary rationale, and the approved ways to communicate with the practice

Explanation: Social media expectations can affect privacy, boundaries, and communication and therefore should be clarified before services. Accepting a personal connection or ignoring the misunderstanding would conflict with the stated practice policy.

Question 20 — Correct: A

Correct Answer: Describe the meaningful variation among domains, the limits of the composite, and how results fit with history and functioning

Explanation: A single composite can obscure substantial and clinically relevant variation among cognitive domains. Results should be explained as a pattern and integrated with other evidence rather than reduced to either the overall or lowest score.

Question 21 — Correct: B

Correct Answer: Construct validity

Explanation: Strong convergence with established measures of the same construct and weak relationships with unrelated constructs support construct validity. These correlations do not assess stability over time or agreement between raters.

Question 22 — Correct: A

Correct Answer: Attention and concentration

Explanation: Reciting months backward requires sustained attention, working concentration, and mental tracking. It does not primarily test distant autobiographical memory or awareness of illness.

Question 23 — Correct: C

Correct Answer: Review the client's technology, privacy, clinical needs, and available formats, then determine whether telephone, periodic in-person, or another referral is suitable

Explanation: Modality selection should integrate practical access with privacy, clinical appropriateness, and emergency planning. Unreliable broadband does not automatically end the referral, but telephone service should not be promised before suitability is assessed.

Question 24 — Correct: C

Correct Answer: Correlating scores from two comparable halves of the same instrument

Explanation: Split-half reliability estimates internal consistency by comparing performance on equivalent portions of one measure. External outcomes concern validity, while two clinician ratings concern interrater reliability.

Question 25 — Correct: A

Correct Answer: Animal-assisted work is optional, its relevant benefits and risks, and the available counseling alternative without an animal

Explanation: The client should understand that the modality is optional and that a nonanimal alternative is available. Requiring exposure before assessment would misrepresent the service and prevent informed choice.

Question 26 — Correct: A

Correct Answer: Explain the record-retention policy, what is documented, applicable access and correction procedures, and the limits of the client's deletion request

Explanation: Record practices and the limits of client control are material to informed expectations. The counselor should not promise deletion that conflicts with the stated policy or withhold information about documentation.

Question 27 — Correct: A

Correct Answer: Explore the client's own concerns and goals, clarify the referral and possible communication, and obtain specific permission before contacting the physician

Explanation: A referral source does not replace the client's account or determine counseling goals automatically. Communication with the physician should be explained and authorized rather than assumed.

Question 28 — Correct: B

Correct Answer: Selective mutism

Explanation: The child consistently speaks in one setting but not at school for longer than one month despite knowing the language and having no speech disorder. The pattern and associated anxiety support selective mutism rather than a global language or social-communication condition.

Question 29 — Correct: A

Correct Answer: 68%

Explanation: In an approximately normal distribution, about 68% of observations lie within one standard deviation of the mean. About 95% lie within two standard deviations.

Question 30 — Correct: A

Correct Answer: Excoriation disorder

Explanation: Recurrent skin picking causing lesions, repeated unsuccessful attempts to stop, and significant distress support excoriation disorder under the supplied exclusions. The central behavior is picking rather than preoccupation with a perceived appearance defect.
